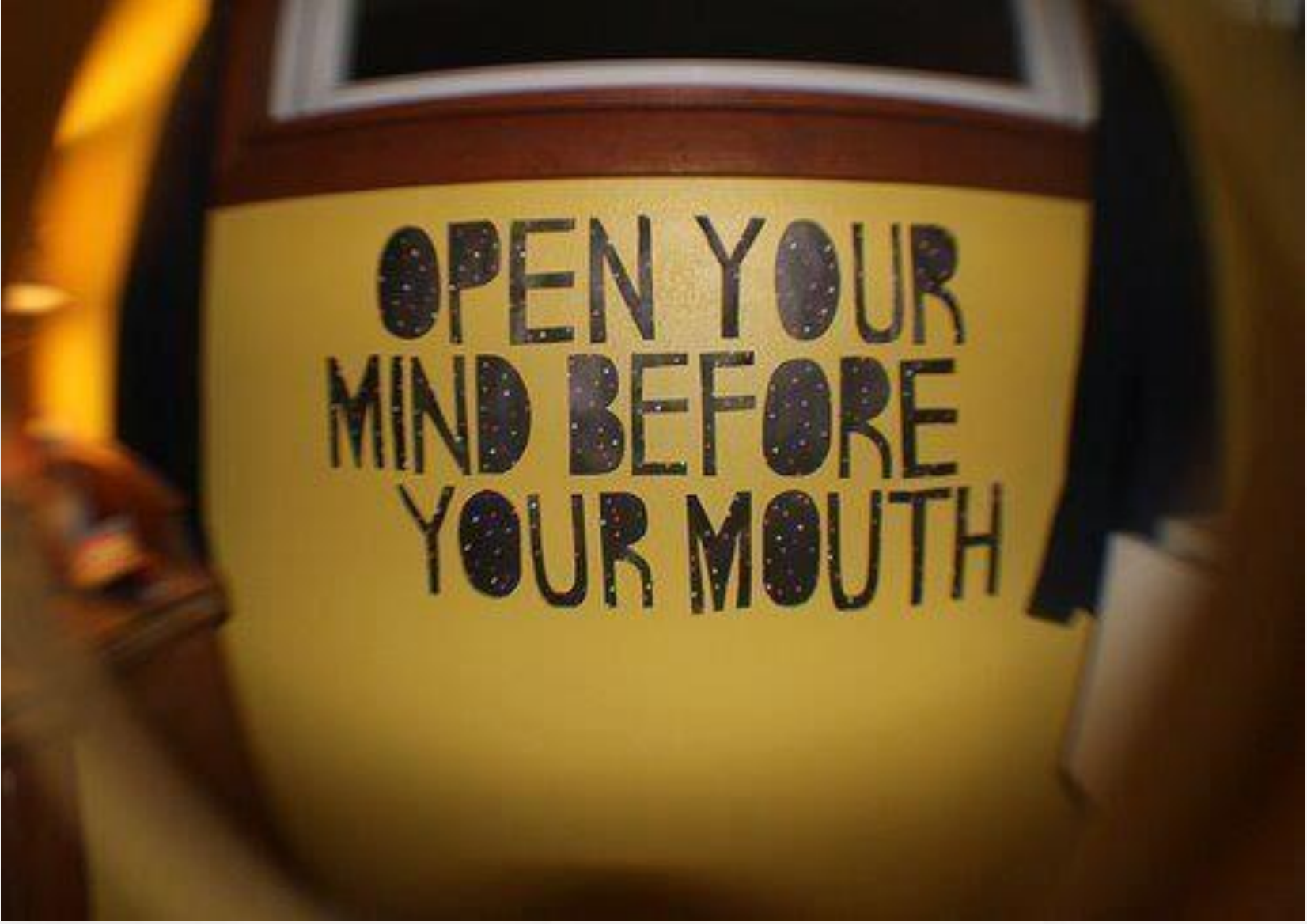


Schizophrenia and Other Psychotic Disorders

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A close-up photograph of a yellow, curved surface, possibly a sign or a piece of equipment. The surface is covered in a pattern of small, dark, star-like specks. In the center, the words "OPEN YOUR MIND BEFORE YOUR MOUTH" are printed in a bold, black, sans-serif font, arranged in three lines. The background is dark and out of focus, with some warm, yellowish light visible on the left side.

**OPEN YOUR
MIND BEFORE
YOUR MOUTH**

Nature of Schizophrenia and Psychosis:

- Schizophrenia vs. Psychosis
 - Psychosis – Broad term (e.g., hallucinations, delusions)
 - Schizophrenia – A type of psychosis
 - Psychosis and Schizophrenia are heterogeneous
 - Disturbed thought, emotion, behavior

Definition

- The schizophrenic disorders are characterized in general by fundamental and characteristic distortions of thinking and perception, and affects that are inappropriate or blunted.
- Clear consciousness and intellectual capacity are usually maintained although certain cognitive deficits may evolve in the course of time.

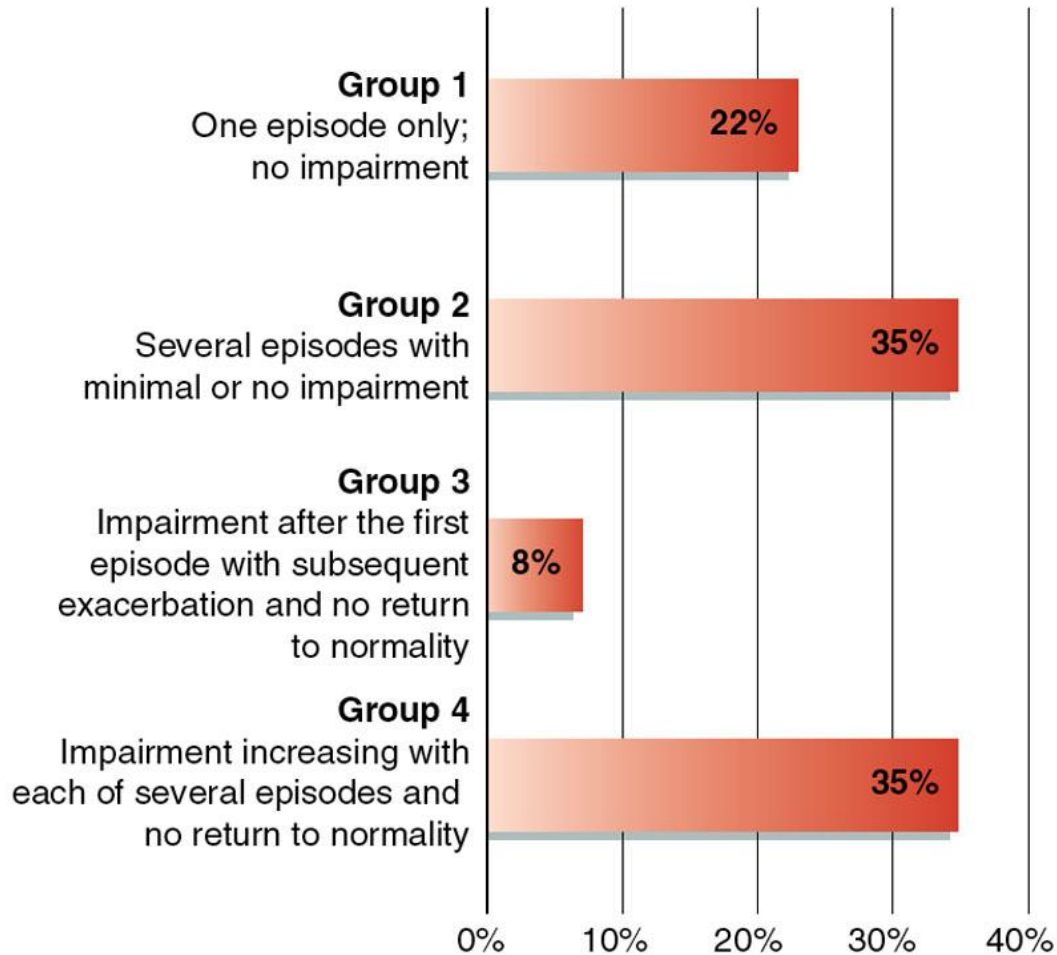
Nature of Schizophrenia and Psychosis

- Historical Background
 - Benedict Morel – Introduced dementia praecox
 - Demence (loss of mind) precoce (early, premature)
 - Emil Kraepelin – Used the term dementia praecox
 - Focused on subtypes of schizophrenia
 - Eugen Bleuler – Introduced the term “schizophrenia”
 - “Splitting of the mind”
 - Kurt Schneider – He emphasized the role of psychotic symptoms, as hallucinations, delusions and gave them the privilege of „the first rank symptoms” even in the concept of the diagnosis of schizophrenia

Schizophrenia: Some Facts and Statistics

- Onset and Prevalence of Schizophrenia worldwide
 - About 0.2% to 1.5% (or about 1% population)
 - Often develops in early adulthood
 - Can emerge at any time
- Schizophrenia is generally chronic
 - Most suffer with moderate-to-severe lifetime impairment
 - Life expectancy is slightly less than average
- Schizophrenia affects males and females about equally
 - Females tend to have a better long-term prognosis
 - Onset differs between males and females
- Schizophrenia has a strong genetic component

Schizophrenia: Some Facts and Statistics (cont.)



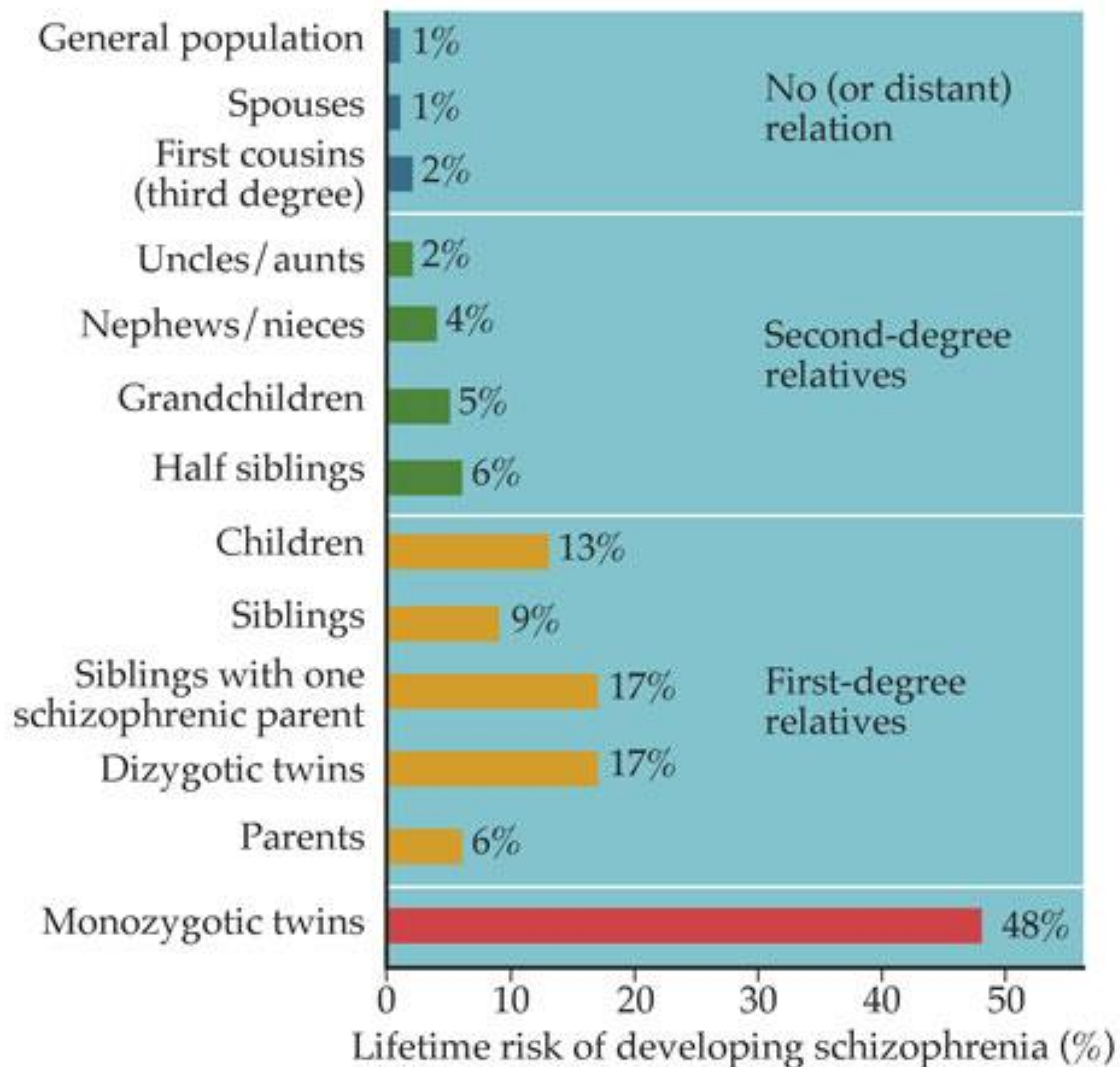


TABLE 16.2 *Features of Schizophrenia*

Positive symptoms	Negative symptoms
Hallucinations, most often auditory	Social withdrawal
Delusions of grandeur, persecution, etc.	Flat affect (blunted emotional responses)
Disordered thought processes	Anhedonia (loss of pleasurable feelings)
Bizarre behaviors	Reduced motivation, poor focus on tasks
	Alogia (reduced speech output)
	Catatonia (reduced movement)

Schizophrenia: The “Positive” Symptom Cluster

- The Positive Symptoms
 - Active manifestations of abnormal behavior
 - Distortions of normal behavior
- Delusions: The basic feature of psychosis
 - Gross misrepresentations of reality
 - Include delusions of grandeur or persecution
- Hallucinations: Auditory and/or Visual
 - Experience of sensory events without environmental input
 - Can involve all senses

Schizophrenia: The “Negative” Symptom Cluster

- The Negative Symptoms
 - Absence or insufficiency of normal behavior
- Spectrum of Negative Symptoms
 - Avolition (or apathy) – Lack of initiation and persistence
 - Alogia – Relative absence of speech
 - Anhedonia – Lack of pleasure, or indifference
 - Affective flattening – Little expressed emotion
 - Asociality – Isolation from public

Schizophrenia: The “Disorganized” Symptom Cluster

- The Disorganized Symptoms
 - Include severe and excess disruptions
 - Speech, behavior, and emotion
- Nature of Disorganized Speech
 - Cognitive slippage – Illogical and incoherent speech
 - Tangentiality – “Going off on a tangent”
 - Loose associations – Conversation in unrelated directions
- Nature of Disorganized Affect
 - Inappropriate emotional behavior
- Nature of Disorganized Behavior
 - Includes a variety of unusual behaviors
 - Catatonia – Spectrum
 - Wild agitation, waxy flexibility, immobility

Course of Illness

- Course of schizophrenia:
 - continuous without temporary improvement
 - episodic with progressive or stable deficit
 - episodic with complete or incomplete remission
- Typical stages of schizophrenia:
 - prodromal phase
 - active phase
 - residual phase

Subtypes of Schizophrenia from DSM IV

- Paranoid Type
 - Intact cognitive skills and affect
 - Do not show disorganized behavior
 - Hallucinations and delusions – Grandeur or persecution
 - The best prognosis of all types of schizophrenia
- Disorganized Type (Hebephrenic)
 - Marked disruptions in speech and behavior
 - Flat or inappropriate affect
 - Hallucinations and delusions – Tend to be fragmented
 - Develops early, tends to be chronic, lacks remissions

Subtypes of Schizophrenia (cont.)

- Catatonic Type
 - Show unusual motor responses and odd mannerisms
 - Examples include echolalia and echopraxia
 - Tends to be severe and rare
- Undifferentiated Type (Atypical Schizophrenia)
 - Wastebasket category
 - Major symptoms of schizophrenia
 - Fail to meet criteria for another type
- Residual Type
 - One past episode of schizophrenia
 - Continue to display less extreme residual symptoms

Schizophrenia Subtypes

Types of Schizophrenia

Paranoid

- Delusions of grandeur or persecution
- Hallucinations (especially auditory)
- Higher level of functioning between episodes
- May have stronger familial link than other types

Disorganized

- Disorganized speech and/or behavior
- Immature emotionality (inappropriate affect)
- Chronic and lacking in remissions

Catatonic

- Alternating immobility and excited agitation
- Unusual motor responses (waxy flexibility, rigidity)
- Odd facial or body mannerisms (often mimicking others)
- Rare

Residual

- Person has had at least one schizophrenic episode but no longer shows major symptoms.
- Still shows “leftover” symptoms (social withdrawal, bizarre thoughts, inactivity, flat affect)

Undifferentiated

- Symptoms of several types that taken together do not neatly fall into one specific category

DSM–V diagnostic criteria for Schizophrenia

1. *Two of the following for most of 1 month;*
 - Delusions
 - Hallucinations
 - Disorganized speech
 - Grossly disorganized or catatonic behavior
 - Negative symptoms
2. *Marked social or occupational dysfunction*
3. *Duration of at least 6 Months of persistent symptoms*
4. *Symptoms of Schizoaffective & mood disorder are ruled out*
5. *Substance abuse & medical conditions are ruled out as aetiological*

